



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ايمان عباس عباس حسنين
المحافظة : القاهرة
الحى / المدينة : حلوان
رقم البطاقة : 28109151701423 رقم التليفون : 01153377496
العنوان : مساكن السكه الحديد
العمر (تاريخ الميلاد) : 15-09-1981 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ايمان عباس عباس حسني National ID: 28109151701423	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٦/٠٣ ٢٨١.٩١٥١٧.١٤٢٣
 حصله على دبلوم صناعة

أنثى مقيمة متزوجة
 محمود عبدالمعصود عبدالستار ابراهيم
 البطاقة سارية حتى ٢٠٢٢/٠٣/٠٩ ١



جمهورية مصر العربية
 بطاقة تحقيق الشخصية

ايمن
 عباس عباس حسنين
 ٢ ش عبود - ش ترعة البوصة الجزيرة
 مركز بنى سويف - بنى سويف

١٩٨١/٠٩/١٩ ٢٨١.٩١٥١٧.١٤٢٣
 LQ8887880





جهة الاصدار : فرع بنى سويف

تاريخ الاصدار : 2022-12-25

الى من يهمه الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال فريق منظمة للاحتفاع الخاضعين لقانون التأمين
الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة
والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم
86 لسنة 2012 " الاطفال دون السن الممتدة " وقانون رقم 23 لسنة 2012 الخاص
بالمرأة المعيلة .

وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية

وطبقا لقاعدة البيانات المرسلة من التأمين الصحي

والبحت تبين ان

المسيد (5) ايمان عباس بطاقة رقم 28109157701423

وطبقا الى خطاب من التأمينات الاجتماعية يفيد ان التأمين
يعتبر من القناتة غير المتكفلة بالتأمين الاجتماعي
ومعها

تاريخ الطباعه : 2022-12-25

الموظف المختار

جمال ابو بكر



القاعدة المركزية للمعلومات
بجدة

مستمل 279495

يعتبر هذا الخطاب ساري لمدة شهر من تاريخ الاصدار
الإدارة المركزية لتكنولوجيا المعلومات ودعم القرار

Patient Name: Eman Abbas Abbas Hasanain	Date : 22/10/2022
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left BC, on neoadjuvant chemotherapy

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Left upper central spiculated lesion harboring suspicious microcalcifications, with anterior group of microcalcifications, measuring collectively 4.3x2.5 cm, seen 2.1 cm from the nipple. US showed spiculated hypoechoic solid mass with internal echogenic shadowing tail. The anterior heterogeneous parenchyma, is not seen in today study
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Bilateral Retroareolar prominent ducts with internal clear contents
- Normal skin thickness and nipple-areola complex.
- Bilateral non-specific axillary LNs.

Opinion:

- Left upper central pathologically proved malignant mass with malignant microcalcifications (partial response) (BIRADS 6)
- Normal right breast study (BIRADS 1)

Yours truly

Signature Dr. Yasmin Nada

22/10/2022 10:54 AM



Patient Name : الأستاذة / ايمان عباس حسان

Patient No. : 28109151701423

Path No. : 3500/22

Sex : Female

Age : 41 Y

Referring Doc :

Request No. : 13009439

Request Date : 17/11/2022

Verification Date : 28/11/2022

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : Ali mostafa

Pathology Report

Clinical Data: Left breast cancer post NACT

Nature of the specimen: Left simple mastectomy & Left SLNB

Gross Examination:

- Procedure: Left SM specimen measured 17x14x5 cm with covering skin ellipse measured 14x10cm
- Tumor extent : An ill defined rubbery tumour deep site
- Size: 12x9x3 cm
- Skin involvement: Nipple & areola were unremarkable grossly
- Distance from margins: 3cm away from deep margin, 1.5 cm away from the skin.
- Lymph nodes number and size: 5 SLNs up to 1x1 cm

Microscopic examination:

- Near complete pathological response, Tumor necrosis index 97%. (<3mm residual component) with fibroadenosis and sclerosing adenosis.
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/5)
- Resection Margins: Free margins
- Histologic response to therapy: Near complete pathological response

Diagnosis:

Left simple mastectomy & Left SLN biopsy (post NACT):

- Near complete pathological response (<3mm residual component)
- Free resection margins
- Free nipple and areola
- Free SLNs (0/5)
- Stage : ypT1a , ypN0 .

Verified by :

Dr.Marwa mahmoud

Printing Date : 03/12/2022 13:07:11

Page 1 of 2

Printing By : ,marwa mohamed youss

مركز الباثولوجي - معمل الباثولوجي - مستشفى عين شمس - القاهرة - مصر
Room No. 401 - First Floor Building No. 401 - Eye Center - Ain Shams University - Cairo, Egypt

Patient Name: Eman Abbas Abbas Hasanain	Date : 09/11/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM in

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C).
- No spiculated masses or grouped microcalcifications
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hilum and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah
09/11/2023 1:29 PM



GA

MDT FORM

15/6/2022

Name	أحمد محمد علي	MRN	1957/15
Age	65	ID	581.91011/1558
Menopausal status	Post	Phone	110 2257297
OCP		Marital status	
Co morbidity		Family history	

Clinical Data

Bx:	IDC 9II							
ER:	8/8	PR:	8/8	HER2:		-ve	Ki67:	10%
Sonomammography:								
Breast Multiplex upper midline masses BIRADS 6								
Mets Workup:								
CT (CP → Abdomen)								
Recommendations:								

Obac scan
Otdo
O Lab

MDT OUTCOME & PLAN

In view of multiple retro-areolar nodules & microcalcifications
→ pt will be for skin sparing mastectomy.

For: NACT (DEC XB → to be done X3) + axilla
He for Ct. SSIT & SLNB +/- reconstruction.

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

C. CC 17/10 C. CC 17/10

SAIV

Patient Name: Eman Abbas Abbas Hasanain	Date : 29/11/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of managed left breast cancer, compared to the last CT examination dated 18/12/2024 (hard copy with no DICOM series), the current examination revealed:

I- CHEST:

- Left MRM with clear operative bed with no evidence of focal lesion or fluid collection.
- Still noted middle lung lobe atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The spleen is average in size and showing multiple calcific foci. Likely granulomas.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

Patient Name: Eman Abbas Abbas Hasanain	Date : 29/11/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phelboli
- No ascites.
- The examined skeleton showed no worrisome lytic or sclerotic lesions.

Opinion:

- Stationary course since the last CT examination regarding middle lung lobe atelectatic bands.
- Multiple splenic calcific foci likely granulomas.

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

Signature Dr.Mohamed Hassan
02/12/2025 4:29 PM



Patient Name: Eman Abbas Abbas Hasanain	Date: 18/12/2024
Exam: C.T CHEST	Age: 42 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS RECONSTRUCTION: -**

I- Chest:

Compared to the study dated 16/11/2022; the new one revealed:

- Left mastectomy. No sizable operative bed masses or enlarged ipsilateral lymph nodes.
- Fine middle lobe atelectatic band otherwise clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II- Abdomen:

Compared to the study dated 25/5/2022; the new one revealed:

- The liver is of average size with homogenous CT texture. Still no focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen. Still seen multiple calcific foci seen within it (likely old granulomatous sequel).
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

Patient Name: Eman Abbas Abbas Hasana'n	Date : 25/05/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS WITH CORONAL RECONSTRUCTION: -**

Chest:

- Multiple left breast enhancing nodules.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Reviewing skeletal parts revealed no lytic or sclerotic osseous lesions.

OPINION:

- Multiple left breast enhancing nodules.
- No evidence of pulmonary nodules or hepatic focal lesions.
- No evidence of lytic or sclerotic osseous lesions

DR. Ibrahim Elsedawy. M.Sc, FEBR.

Dr. M. Gomaa, M.D.

Name: Mrs. iman abbas abbas

2025/12/6

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post left MRM routine follow up.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma . (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.,
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

- *Right breast (BIRADS 1): normal study.*
- *Clear left mastectomy bed and axilla.*

Dr farah elshobaky

Patient Name: Eman Abbas Abbas Hasanain	Date : 29/11/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

**MULTISCLICE NON CONTRAST C.T. EXAMINATION OF THE
CHEST, ABDOMEN AND PELVIS REVEALED:**

Known case of managed left breast cancer, compared to the last CT examination dated 18/12/2024 (hard copy with no DICOM series), the current examination revealed:

I- CHEST:

- Left MRM with clear operative bed with no evidence of focal lesion or fluid collection.
- Still noted middle lung lobe atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The spleen is average in size and showing multiple calcific foci. Likely granulomas.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

Patient Name: Eman Abbas Abbas Hasanain	Date : 29/11/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phelboli
- No ascites.
- The examined skelcton showed no worrisome lytic or sclerotic lesions.

Opinion:

- Stationary course since the last CT examination regarding middle lung lobe atelectatic bands.
- Multiple splenic calcific foci likely granulomas.

Dr. Norhan Ramzy, MD

Dr. NOURHAN RAMZY

Dr. NOURHAN RAMZY

Signature Dr. Mohamed Hassan
02/12/2025 4:29 PM

SIGNATURE				
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition

MDT OUTCOME & PLAN

For ① Adjuvant Tamoxifen
② PORT.

Recommendations:

free margins
SLNB - 10/15
Tumor complete pathology

Mets Workup:

Sonomammography:

Post-ILSM + SLNB

ER:	818	PR:	818	HER 2:	-ve	Ki67:	100%
-----	-----	-----	-----	--------	-----	-------	------

BX: IDC G-II

Clinical Data

Name		- 1952-52-1	
Age		41	
Menopausal status		premenopausal	
NRS SCORE		7	
BMI		20.5	
CO morbidity		NAD	
MRN	1952-52-1	ID	CA19101V1624
Phone	-1104222397	Marital status	me
Family history	me	REASON OF MDT	

MDT FORM

15/12/2022 G



وزارة الصحة العامة
مملكة العربية السعودية



جمهورية مصر العربية
وزارة الداخلية
مصلحة الأحوال الشخصية

إيمان
عباس عباس حسين
مستلقة التسمية التخليد لزوجها عزيزة زين
خلوان - القاهرة

١٩٨١/٥/١٥
٢٨١.٩١٥١٧.١٤٢٣

HV7243354

٢٠١٩/٠١ ٢٨١.٩١٥١٧.١٤٢٣
حصله على دبلوم صناعة

انتفى
محمود عبدالمعروف عبدالتيسر إبراهيم
البطاقة سارية حتى ٢٠٢٦/٠٢/٠٥

١



ERM036722691



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

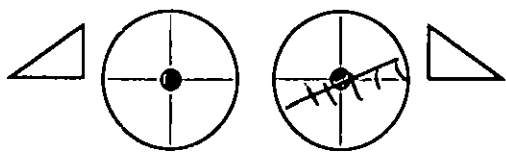
-١٩٨٢٨-٩٢-٨

MDT FORM

15/12/2022 G

Name	إيمان محمد علي	MRN	1987-9-25
Age	41	ID	9810910171422
Menopausal status	premenopausal	Phone	-1103377497
NRS SCORE	2	Marital status	
BMI	obese	Family history	me
CO morbidity	NAD	REASON OF MDT	

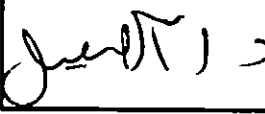
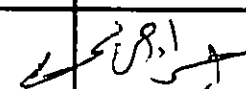
Clinical Data

BX:	IDC GII	R	L
ER:	818	PR:	818
HER 2:	-ve	Ki67:	10%
Sonomammography:	Post L+SM + SLNB		
Mets Workup:	free margins SLNB - D015	TNM	
Recommendations:	Clear margins Pathology at R		

MDT OUTCOME & PLAN

For ① Adjuvant Tamoxifen
② PORT.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

SMC-DSH-F-MDT-001

Revision no:0

Issue no :1

Issue date: 1/9/2022

MDT Form

[[DOCTYPE:MDT_FORM]]



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
صحة
مصر

G

MDT FORM

15/6/2022

Name	ايمن عيسى	MRN	1997/15
Age	45	ID	281,91,017,1422
Menopausal status		Phone	110 2377 497
OCP		marital status	
Co morbidity		Family history	

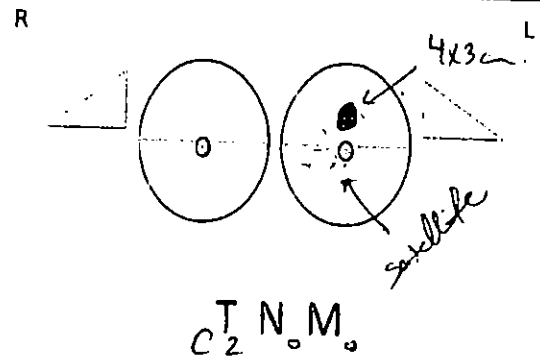
Clinical Data

Bx: IDC GII

ER: 8/8 PR: 8/8 HER2: -ve Ki67: 10%

Sonomammography:
Breast Multiphopper
midline masses. BIRADS 6

Mets Workup:
CT CAP → No met,



Recommendations:

- ① Bas scan
- ② tdo
- ③ Labs

MDT OUTCOME & PLAN

In view of multiple retro-areolar satellites & microcalcifications
→ ~~the~~ will be for skin sparing mastectomy.

For: NACT (FEC X3 → taxane X3) ⊕ 7 cycles
then for Rt. SSF & SLNB +/- reconstruction.

SIGNATURE

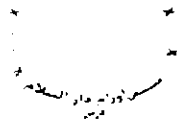
MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

C. CC / 7/10 C. CC / 7/10



Patient Name: Eman Abbas Abbas Hasanain	Date : 22/10/2022
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left BC, on neoadjuvant chemotherapy

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Left upper central spiculated lesion harboring suspicious microcalcifications, with anterior group of microcalcifications , measuring collectively 4.3x2.5 cm, seen 2.1 cm from the nipple. US showed spiculated hypoechoic solid mass with internal echogenic shadowing foci. The anterior heterogeneous parenchyma, is not seen in today study
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Bilateral Retroareolar prominent ducts with internal clear contents
- Normal skin thickness and nipple-areola complex.
- Bilateral non-specific axillary LNs.

Opinion:

- Left upper central pathologically proved malignant mass with malignant microcalcifications (partial response) (BIRADS6)
- Normal right breast study (BIRADS 1)

Yours truly

Signature Dr. Yasmin Nada

22/10/2022 10:54 AM



Patient Name: Eman Abbas Abbas Hasanain	Date : 18/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical 'D: 1927/2022	Pat. ID: 1760

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

18/12/2024 12:10 PM

PATIENT NAME: TAGRED AHMED IBRAHIM SHAMES EL DEEN	PATIENT ID #: 2021159730
GENDER: Female	ORDER #: 1129077,1129078
AGE: 46 y	DATE OF SERVICE: 04-Jul-2021 03:13 PM

Report: - all studies

**DIGITAL MAMMOGRAPHY OF BOTH BREASTS & COMPLEMENTARY
ULTRASOUND**

Clinical indication : palpable right breast mass lesion.

- ACR C; Heterogenous fibroglandular breast parenchyma

Right breast :

- **UOQ** is the seat of irregular, lobulated mass lesion with overlying scattered microcalcifications.
- Scattered non grouped rounded microcalcifications are noted.
- Complementary US shows
- Concordant with the mammographic findings & palpable lump there is 1.00 large heterogeneously hypoechoic irregular mass, showing surrounding echogenic halo & measuring about 45 x 45 x 28 mm in maximum diameters.
- Surrounding the lesion (especially from 11- 1 o'clock there is heterogenous altered parenchymal echogenicity).
- Retroareolar & para areolar prominent ducts showing isoechoic contents extending from the nipple to the mass suggesting intraductal extension.
- Moderate appreciable vascularity noted within the lesion.
- The mass located 8 mm deep to the skin & 29mm away from the nipple/areolar complex
- Normal contour and skin thickness.
- Normal nipple- areola complexes.
- Enlarged axillary lymph nodes one of the in lateral axilla showing focal cortical thickening (3.8mm)

Left breast:

- No speculated mass lesions
- Scattered microcalcifications noted, few of them are grouped at roughly upper central quadrant, this was confirmed by US as 12.00 few irregular anechoic few cystic lesions.
- UOQ area of palpable adenosis with small hypoechoic suspicious nodule measuring about 5mm.

→ hypoechoic

PATIENT: TAGRED AHMED IBRAHIM SHAMES EL DEEN, 2021159730, Female, 46 y
Report version 1, 18-Jul-2021 03:39 PM
Page 1 of 2

Name : ايمان عباس عباس حسنين

Ref. By Prof. Dr. : مركز هارمیل

Date : 19/06/2022

Code No. : 11983273

^{99m}Tc-MDP W.B. Bone Scan**History:**

- 40 years old female patient with history of breast neoplasm.

Technique:

- Whole body images were acquired 3 hours after injection of 740 MBq of the tracer.

Findings:

- No features of osseous metastases detected.
- Mild degenerative changes of the spine.

OPINION:

- No scintigraphic evidence of osseous metastases.

*Much obliged,
Dr. Alaa Nagi, MD
Nuclear Medicine*



Patient Name: Eman Abbas Abbas Hasanain	Date : 25/05/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS WITH CORONAL RECONSTRUCTION: -**

Chest:

- Multiple left breast enhancing nodules.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Reviewing skeletal parts revealed no lytic or sclerotic osseous lesions.

OPINION:

- **Multiple left breast enhancing nodules.**
- **No evidence of pulmonary nodules or hepatic focal lesions.**
- **No evidence of lytic or sclerotic osseous lesions**

DR, Ibrahim Elsedawy.M.Sc, FEBR

Dr. M. Gomaa, M.D.



Women's Health Outreach Program

Name: eman abbas abbas

Date: 31.05.2022

Contrast Enhanced Full field Digital Mammography

Clinical summary:

left breast IDC grade II.

Technique

Pre & post contrast Mediolateral oblique and CC views

Targeted High resolution ultrasound

Detailed Findings

Left breast

- Pre-contrast images revealed: multiple ill-defined masses at the upper central.
- Contrast enhanced images showed:
- There is ill defined rounded heterogeneous mass enhancement located at 12 o'clock, 3 cm from the nipple, measure 3 x 3 x 2 cm in diameter.
- Similar smaller lesions are seen anterior to the index mass, their diameter ranging 7-9 mm and the most anterior one is 1 cm from the nipple, all together measured 8 x 6 cm.
- Normal skin/nipple/areolar complex.
- Ultrasound reported No pathological axillary LNs.

Right breast

- Pre-contrast images revealed: no suspicious masses.
- Contrast enhanced images demonstrated:
- No mass or non-mass enhancement.
- Normal skin/nipple/areolar complex.
- No pathological axillary LNs.

Conclusion:

-Left breast: multiple upper midline pathological enhancing masses (BIRADS 6).

-Right breast: normal study (BIRADS 1).

Best regards,
Hanaa Abo El Magd



Women's Health Outreach Program

Name: Eman Abass Hassanien

Date: 26-04-2022

Full field Bilateral Digital Mammography & Ultrasonography

Clinical summary: 40 year old female with palpable left breast lump of 1 week duration.

Detailed Findings

Technique

Craniocaudal&meciolateral oblique views

High resolution ultrasound

Left breast

- Extremely dense breast that lowers mammographic sensitivity (ACR d).
- Left Upper central mass density lesion, showing irregular shape and speculated margins with suspicious microcalcifications.
- Two other groups of microcalcifications are noted along a line extending from such mass density lesion and the nipple.
- Ultrasound demonstrated.
index mass: irregular hypoechoic with speculated margin seen at 11-12 o'clock; 4 mm from the skin and 25 mm from the nipple; it measured 36 x 21 mm with moderate vascularity on Color Doppler study. It showed possible ductal extension.
The parenchyma between the index mass and the nipple showed heterogeneity and nodularity with two definite satellites are noted at 12 o'clock position; one measuring 7 x 12 mm , 4 mm from the skin and 7.5 mm from the index mass. the other measured 8 x 9 mm seen 12 mm from the previous nodule ; seen retroareolar. both nodules showed mild vascularity on colour Doppler study with ductal extension.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Right breast

- Extremely dense breast that lowers mammographic sensitivity (ACR d).
- No masses, asymmetric densities , distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions
- Normal skin/nipple/areolar complex.
- non-specific axillary LNs .

Conclusion:

-Left breast: (BIRADS 5); picture is suggestive of multifocal malignant process, as described .

-Right breast: (BIRADS 1), normal study.

Recommendation

Left breast: biopsy and histopathological correlation.

Right breast : annual screening.

Best regards,

Dr. Amal Hegaqzi, M.Sc.

Dr. Rania Ahmed., M.D.



Patient Name: Eman Abbas Abbas Hasanain	Date 18/12/2024
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS RECONSTRUCTION: -**

I- Chest:

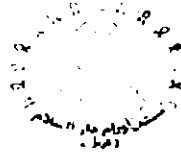
Compared to the study dated 16/11/2022; the new one revealed:

- Left mastectomy. No sizable operative bed masses or enlarged ipsilateral lymph nodes.
- Fine middle lobe atelectatic band otherwise clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II- Abdomen:

Compared to the study dated 25/5/2022; the new one revealed:

- The liver is of average size with homogenous CT texture. Still no focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen. Still seen multiple calcific foci seen within it (likely old granulomatous sequel).
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.



Patient Name: Eman Abbas Abbas Hasanain	Date : 18/12/2024
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 1927/2022	Pat. ID: 11760

- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Reviewing skeletal parts revealed no lytic or sclerotic osseous lesions.

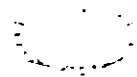
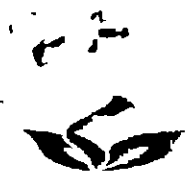
OPINION:

Follow up status showing:

- Status post left mastectomy showing no operative bed masses.
- Still no evidence of pulmonary or hepatic metastatic lesions.

Dr.Shaimaa Megahed,MSc

~~Dr. Mohamed Mofsen, M.D.~~



Patient Name	: الأستاذة / ايمان عباس عباس حسنين	Request No.	: 13000110
Patient No.	: 28109151701423	Request Date	: 27/04/2022
Path No.	: 1136/22	Verification Date	: 21/05/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Dcc.	:	IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of the specimen: Tru cut biopsy.

Gross Examination:

Four grayish white tissue core biopsy measured 1 cm each, totally submitted.

• Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- Histological type Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1).
- Ductal carcinoma in situ: Focal.
- IPT :

ER: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8)

Her-2: No membrane reaction in tumor cells (score 0)

Ki 67: 10 % nuclear reaction

Diagnosis:

Left breast mass, Tru cut biopsy:

- Invasive duct carcinoma GII.
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- HER-2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Prof. Dr. Gamal Abdellaty.

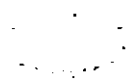
Verified by :

Dr. Marwa mahmoud

Printing Date : 21/05/2022 13:38:26

Page 1 of 2

Printed By : Ali mostafa



Patient Name : الأستاذة / ايمان عباس عباس حنين	Request No. : 13009439
Patient No. : 28109151701423	Request Date : 17/11/2022
Path No. : 3500/22	Verification Date : 28/11/2022
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Left breast cancer post NACT

Nature of the specimen: Left simple mastectomy & Left SLNB

Gross Examination:

- Procedure: Left SM specimen measured 17x14x5 cm with covering skin ellipse measured 14x10cm
- Tumor extent : An ill defined rubbery tumour bed site
- Size: 12x9x3 cm
- Skin involvement: Nipple & areola were unremarkable grossly
- Distance from margins: 3cm away from deep margin. 1.5 cm away from the skin.
- Lymph nodes number and size: 5 SLNs up to 1x1 cm .

Microscopic examination:

- Near complete pathological response, Tumor necrosis index 97%. (<3mm residual component) with fibroadenosis and sclerosing adenosis.
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/5)
- Resection Margins: Free margins
- Histologic response to therapy: Near complete pathological response

Diagnosis:

Left simple mastectomy & Left SLN biopsy (post NACT):

- Near complete pathological response (<3mm residual component)
- Free resection margins
- Free nipple and areola
- Free SLNs (0/5)
- Stage : ypT1a , ypN0 .

Verified by :

Dr.Marwa mahmoud